

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Orlistat 120mg capsules for the treatment of Weight management in Obesity

Summary of NICE / NICE CKS Guidelines for Weight management in Obesity

Overview of Obesity and Weight Management

- **Definition of Obesity:** BMI ≥ 30 kg/m² (or ≥ 27.5 in some ethnic groups with increased cardiometabolic risk)
- **Associated conditions:** Type 2 diabetes mellitus, cardiovascular disease, obstructive sleep apnoea, musculoskeletal disorders

Assessment

- **Weight and anthropometric measurements:** Calculate BMI, measure waist circumference
- **Comorbidities:** Assess for associated chronic conditions and cardiovascular risk
- **Previous weight management attempts:** Document history and response to lifestyle interventions
- **Current medications:** Review for medications that may affect weight or interact with orlistat
- **Eating habits and lifestyle:** Assess diet quality, physical activity, and readiness for behaviour change

Management

- **First-line intervention:** Diet (reduced-calorie intake), physical activity (at least 150 minutes moderate-intensity per week), behaviour change
- **Pharmacological treatment:** Consider as adjunct to lifestyle changes when BMI ≥ 30 kg/m² OR BMI ≥ 28 kg/m² with associated comorbidities (T2DM, hypertension, dyslipidaemia, CVD)
- **Orlistat:** Pancreatic and gastric lipase inhibitor; reduces absorption of dietary fat

Red Flags (Refer to GP)

- **Unintentional weight loss:** May indicate underlying malignancy or other serious pathology
- **Symptoms suggestive of secondary obesity:** Cushing's syndrome, hypothyroidism, polycystic ovary syndrome, medications (corticosteroids, antipsychotics)

Patient Group Direction

for the supply/administration of

Orlistat 120mg capsules

for the treatment of

Weight management in Obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and
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	requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Adjunct to reduced-calorie diet and lifestyle changes in the management of obesity in adults with BMI ≥ 30 kg/m ² or BMI ≥ 28 kg/m ² with associated comorbidities such as Type 2 diabetes mellitus, hypertension, dyslipidaemia or cardiovascular disease.
Inclusion criteria	Age ≥ 18 years and < 75 years BMI ≥ 30 kg/m² OR BMI ≥ 28 kg/m² with at least one obesity-related comorbidity (T2DM, hypertension, dyslipidaemia, CVD) Motivated and committed to weight loss with structured diet (reduced-calorie diet, typically 500-1000 kcal below estimated daily expenditure) Informed consent obtained; patient understands medication mechanism, expected outcomes, and common side effects Baseline weight and waist circumference documented
Exclusion criteria	Chronic malabsorption syndrome (e.g., cystic fibrosis, coeliac disease, inflammatory bowel disease) Cholestasis or severe hepatic impairment Pregnancy or breast-feeding Concurrent ciclosporin therapy (reduced absorption of ciclosporin) Warfarin therapy (relative contraindication; requires specialist assessment) Known hypersensitivity to orlistat or any component of the

	formulation Age <18 years or ≥75 years Uncontrolled or newly diagnosed diabetes (require GP review before starting)
Cautions	Diabetes mellitus: Blood glucose control may improve; dose of antidiabetic medication (especially insulin and sulfonylureas) may require adjustment Anticoagulant therapy (warfarin, edoxaban, dabigatran, rivaroxaban): Enhanced anticoagulant effect; requires GP liaison and INR monitoring if applicable Fat-soluble vitamin absorption: May be reduced (vitamins A, D, E, K); recommend multivitamin supplementation at least 2 hours apart from orlistat Hypothyroidism: Levothyroxine absorption may be reduced; monitor thyroid function and dose; administer levothyroxine at least 4 hours before orlistat History of oxalate kidney stones: Risk of hyperoxaluria and recurrence Chronic liver disease or elevated liver function tests: Proceed with caution; ensure baseline LFTs checked Gallstone disease or bile acid sequestrants: Monitor for interactions and symptoms
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Orlistat 120mg capsules
Legal category	POM (Prescription-Only Medicine)
Storage	Store below 25°C. Keep in original container. Protect from moisture.
Route/method of administration	Oral administration; swallow capsule whole with a glass of water with or shortly before each main meal.
Dose and frequency	120mg with each main meal containing fat (up to 3 times daily, typically with breakfast, lunch, and dinner) If a meal is missed or contains negligible fat, omit the dose Maximum: 360mg daily (3 × 120mg)
Quantity to be administered and/or supplied	Up to 84 capsules per patient (28-day supply at maximum dose of 3 capsules daily)

Maximum or minimum treatment period	Review at 12 weeks (3 months) from start of treatment. Continue treatment only if patient has achieved $\geq 5\%$ reduction in body weight from baseline. If weight loss target not achieved, discontinue and refer to GP for alternative management strategies.
Adverse effects	Common ($\geq 1\%$ in clinical trials): Oily spotting, flatulence with discharge (faecal incontinence), faecal urgency, fatty or oily stool, increased defaecation, abdominal pain, headache, urgency of stool defaecation Uncommon: Diarrhoea, upper respiratory tract infection, lower respiratory tract infection, anxiety, fatigue, nausea Rare: Hepatitis, pancreatitis, oxalate nephropathy (hyperoxaluria), cholesterol gallstone formation, rash, pruritus The incidence and severity of GI adverse effects typically decrease over time as patients adjust their fat intake

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: - Adults aged 18 years and over - keep records for 8 years - Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the Patient Information Leaflet (PIL) provided with Orlistat. Ensure patient understands mechanism of action, dietary requirements, and expected side effects.
Follow-up advice to be given to patient or carer	Maintain a reduced-calorie diet with low fat content ($\leq 30\%$ of daily calories from fat) to minimise GI side effects such as oily spotting,

	flatulence, and stool urgency. Take a multivitamin supplement containing fat-soluble vitamins (A, D, E, K) at least 2 hours apart from orlistat to ensure adequate vitamin absorption. Do not take orlistat if a meal is missed or contains no fat (e.g., fat-free meal). If taking levothyroxine, administer at least 4 hours before orlistat to avoid reduced absorption. Report any jaundice, persistent abdominal pain, signs of pancreatitis, or persistent diarrhoea to the GP immediately. Expect weight loss to be gradual (typically 2-4 kg in the first 12 weeks if combined with a reduced-calorie diet and exercise). If diabetes medications are being taken, inform the GP as blood glucose control may improve and medication adjustment may be needed. Attend follow-up appointment at 12 weeks to assess weight loss progress ($\geq 5\%$ target); continue treatment only if this target is met. If on warfarin or other anticoagulant therapy, ensure GP is aware and INR is monitored as appropriate. Report any suspected adverse drug reactions or side effects to the GP or via the Yellow Card scheme at https://yellowcard.mhra.gov.uk.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Weight management and obesity
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			

Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025